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NUR 101: Fundamentals of Nursing - Care, Assessment and Safety

The first-year foundations of safe, dignified patient care

FACULTY PRIMER

Faculty Primer

Faculty of Health Sciences & Technology
Nursing Sciences | 100L

Written for students, by students.

Practical notes, worked examples, practice questions and a revision plan.

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Independent study support | Not an official UNIZIK publication

START HERE

Why this pack exists

Nursing fundamentals are assessed both in written papers and in the skills laboratory. This pack explains the nursing process, vital signs, infection prevention and patient safety, and includes a skills-practice log you can bring to demonstrations.

This is a student-written study companion, not a reproduced lecture handout or a leaked examination paper. Use it to organise the topics you are taught, practise the kind of thinking your course rewards, and walk into class with better questions.

What you will be able to do

- Describe the steps of the nursing process and apply them to a simple scenario.
- Measure and interpret temperature, pulse, respiration, blood pressure and oxygen saturation.
- Explain standard precautions and correct hand-hygiene technique.
- Identify common patient-safety risks and the nursing actions that prevent harm.

How to use this pack

1. Skim the whole pack once in 20 minutes so you know what is inside.
2. Work through one section at a time with your lecture note and recommended text beside you.
3. Attempt the practice questions before reading the answers. Mark what you missed.
4. Use the final checklist to plan your revision week, one tick per study session.

> **Free to download, always**
> This pack is free. There is no fee, no subscription and no payment step before download, and the full PDF is not watermarked or locked. Share the link with a coursemate instead of forwarding the file.

> **Confirm your official course outline**
> Course codes, credit units and outlines can differ by department and session. Treat this pack as supplementary support and confirm the current outline with your lecturer, course rep or departmental handbook before an assessment.

The nursing process

Assessment, diagnosis, planning, implementation and evaluation form a cycle, not a linear checklist.

- Assessment: collect subjective and objective data systematically.
- Diagnosis: identify the patient's actual or potential health problems in nursing language.
- Planning: set measurable, patient-centred goals and select interventions.
- Implementation: carry out care safely and document it.
- Evaluation: compare outcomes with goals and revise the plan.
- Documentation is part of care; if it is not recorded, it is treated as not done.

Vital signs and observation

Vital signs are the earliest warning system you have. Accuracy matters more than speed.

- Normal adult ranges: temperature about 36.1 to 37.2 C, pulse 60 to 100 beats per minute, respiration 12 to 20 per minute.
- Blood pressure is recorded as systolic over diastolic; use the correct cuff size.
- Count respirations for a full minute without telling the patient, to avoid altering the rate.
- Report trends, not single readings: a rising pulse with falling pressure is a warning sign.
- Record the time, site and position used for blood pressure measurement.
- Pain is a vital sign and must be assessed with a consistent scale.

> **Escalate early**
> A deteriorating patient rarely crashes without warning. Track early-warning scores and escalate according to your facility's protocol rather than waiting for the next round.

Infection control and safety

Most hospital-acquired infection is preventable with consistent basic practice.

- Standard precautions apply to every patient regardless of diagnosis.
- Hand hygiene before and after patient contact, after body-fluid exposure and after removing gloves.
- Use gloves, aprons, masks and eye protection according to risk; remove them correctly.
- Sharps go straight into a puncture-resistant container; never recap a needle by hand.
- Identify the patient using at least two identifiers before any intervention.
- Prevent falls: call bell in reach, bed at correct height, brakes on, clutter cleared.

The nursing process

Nursing practice is organised through assessment, diagnosis, planning, implementation and evaluation - a problem-solving cycle, not a form. Assessment gathers subjective and objective data; the nursing diagnosis identifies the patient's response to a health problem; planning sets measurable goals; implementation delivers care; evaluation checks whether goals were met.

The distinction between medical and nursing diagnosis is examinable. A medical diagnosis names a disease; a nursing diagnosis names a human response such as impaired comfort or risk of infection. Interventions follow from the response, so two patients with the same disease may need different plans.

Documentation is part of the process. What is not recorded is treated as not done, because the record is the legal account of care. Objective, timed, signed entries protect the patient and the nurse.

Vital signs, hygiene and safety

Vital signs - temperature, pulse, respiration, blood pressure and oxygen saturation - are the earliest indicators of deterioration. Measure correctly, record accurately, and act on abnormal values immediately.

Infection prevention is the single most effective intervention: hand hygiene before and after every contact, correct glove use, aseptic technique for invasive procedures, and safe sharps disposal. Most hospital-acquired infection is preventable and the prevention is unglamorous.

Patient safety includes fall prevention, pressure area care, medication safety and correct identification. The five rights of medication administration exist because each has been the site of fatal error; accurate documentation is the sixth.

> **Escalate early**
> A rising respiratory rate is the most sensitive early sign of deterioration. Report it rather than waiting to be sure.

Communication, ethics and boundaries

Communication is a clinical skill. Open questions gather information, closed questions confirm it, summarising checks understanding, and silence is a tool. Teach-back confirms that instructions were understood.

Ethical practice rests on autonomy, beneficence, non-maleficence and justice. Consent, confidentiality, truthfulness and advocacy follow from them. Advocacy means representing the patient when they cannot.

The therapeutic relationship has boundaries. Kindness is professional; entanglement is not. Cultural sensitivity, privacy and awareness of your own reactions are part of competence.

WORKED EXAMPLES

Do it once, slowly

These are not model answers to copy. They show the reasoning a strong student would use, step by step, so you can repeat the process on a question you have not seen.

1. Writing a care plan

A post-operative patient is reluctant to move and breathes shallowly.

1. Assessment: reduced mobility, shallow respirations, pain on movement, saturation at the lower limit.
2. Nursing diagnosis: ineffective breathing pattern related to pain and immobility.
3. Goal: saturation within normal range and hourly deep breathing.
4. Interventions: prescribed analgesia, positioning, spirometry teaching, hourly reinforcement.
5. Evaluation: record saturation, breath sounds and adherence each shift.
6. State escalation criteria explicitly.

> **What to carry forward**
> A care plan names the response, the goal, the action and the evidence of evaluation.

2. Handling a medication error

You realise you gave a drug an hour late.

1. Assess the patient and take observations.
2. Inform the nurse in charge without delay.
3. Complete an incident report factually, without blame.
4. Record the event and actions in the notes.
5. Inform the prescriber if clinically significant.
6. Reflect on the system cause, not only individual attention.

> **What to carry forward**
> Report, assess, document, learn. Concealment turns an error into misconduct.

EXAM TECHNIQUE

Questions with model answers

Written in the shape your course is usually examined, with the marking logic spelled out. Attempt each one under time before you read the outline.

Q1. Explain the five steps of the nursing process.

15 marks

1. Assessment of subjective and objective data.
2. Diagnosis of the patient's response.
3. Planning of measurable goals.
4. Implementation of interventions.
5. Evaluation against goals, with revision.
6. Note that the process is cyclical.

Q2. Discuss the nurse's role in infection prevention.

15 marks

1. Hand hygiene and its indications.
2. Aseptic technique for invasive procedures.
3. Personal protective equipment.
4. Sharps disposal and waste segregation.
5. Isolation precautions and patient education.

REFERENCE

Glossary

Use these definitions in your own sentences; a copied definition rarely earns full marks.

Nursing process. A five-step problem-solving cycle from assessment to evaluation.

Nursing diagnosis. A statement of a patient's response, distinct from a medical diagnosis.

Aseptic technique. Practices preventing contamination of a site or equipment.

Vital signs. Temperature, pulse, respiration, blood pressure and oxygen saturation.

Tachypnoea. Abnormally rapid breathing, an early sign of deterioration.

Five rights. Right patient, drug, dose, route and time.

Incident report. A factual record of an adverse event for review and learning.

Advocacy. Acting to protect a patient's interests.

Autonomy. The patient's right to make informed decisions about care.

Pressure area care. Interventions preventing skin breakdown in immobile patients.

Teach-back. Asking a patient to restate information to confirm understanding.

Escalation. Referring a deteriorating patient promptly to a senior clinician.

Your four-week plan

Week 1 - Process

Structure.

- Write a care plan for one case.
- Distinguish nursing from medical diagnoses.
- Practise objective documentation.

Week 2 - Observations

Vital signs.

- Record vital signs for five patients.
- State normal ranges.
- Identify two escalation triggers.

Week 3 - Safety

Infection and medication.

- Demonstrate hand hygiene.
- List the five rights.
- Describe sharps disposal.

Week 4 - Communication

Patients.

- Practise open-question interviews.
- Use teach-back.
- Discuss one ethical scenario.

Where to read more

Entry-level texts you can find in the university library or a departmental reading list. Confirm editions with your lecturer.

- Any fundamentals of nursing text covering the nursing process.
- Your clinical skills laboratory procedure checklists.
- Your institution's infection prevention policy.

PRACTICE, NOT PREDICTION

Check your understanding

These questions are written for this pack to test the ideas above. They are not past examination questions and do not predict what your lecturer will set. Answer first, then compare.

1. List the five steps of the nursing process.

Answer: Assessment, diagnosis, planning, implementation and evaluation.

2. Why should respirations be counted without informing the patient?

Answer: Awareness can cause the patient to alter their breathing pattern, producing an inaccurate reading.

3. What are standard precautions?

Answer: Infection-control practices applied to all patients, including hand hygiene, appropriate personal protective equipment, safe sharps handling and safe waste disposal.

4. Give two nursing actions to prevent falls.

Answer: Keep the call bell within reach and the bed at the lowest safe height with brakes locked. Remove clutter and ensure adequate lighting.

5. Differentiate subjective from objective data.

Answer: Subjective data are what the patient reports, such as pain. Objective data are measurable findings, such as temperature or a wound appearance.

Your 7-step action plan

Tick each step only when you have done it. A tick you cannot defend is worse than an empty box.

- [ ] Write the nursing process from memory with one example per step.
- [ ] Practise taking all five vital signs on a partner and record them correctly.
- [ ] Demonstrate hand hygiene using the approved technique.
- [ ] List the sequence for donning and removing personal protective equipment.
- [ ] Identify ten fall and pressure-injury risks in a ward layout.
- [ ] Complete a skills-practice log entry with a supervisor's signature.
- [ ] Read your school's incident-reporting policy and summarise it in five lines.

Keep going

One pack will not carry a semester. Pair this with your lecture notes, tutorials, and the past questions your department actually releases. If a topic still feels unclear, take a specific question to your lecturer or course rep - "I do not understand the difference between X and Y" gets a better answer than "I do not understand this course".

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YOUR WORKSPACE

Notes, questions and things to ask

Use this page in class or during revision. Write questions you could not answer instead of copying slides.

Three ideas I can now explain without notes:

A question I still cannot answer:

For my lecturer or course rep:

Next revision action and date:

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